



Fiona Stanley Fremantle Hospitals Group

Procedure

Baby boarders: Care in the Mother Baby Unit

Reference #: FSFH-MENH-PRO-0004

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Mother Baby Unit	Medical, Nursing

1. Introduction

Mothers are admitted as inpatients to the Mother Baby Unit (MBU) and their babies are registered onto the hospital's patient administration system as boarders. The mother is responsible for the baby's care during hospitalisation. However, despite the baby's status as a boarder, Mental Health clinicians have a duty of care responsibility for the baby. This document describes the care required for babies boarding with their mother in the MBU.

2. Terminology

Baby boarder	infant/baby who is receiving accommodation but for whom the hospital does not accept responsibility for treatment and/or care.
PSOLIS/webPSOLIS	Psychiatric services online information system.
Well baby check	Medical assessment which includes the review and interpretation of the baby's physiological observations pulse, respiratory rate and oxygen saturation if indicated.

3. Procedure

3.1. Referrals to the Mother Baby Unit (MBU)

- The MBU Clinical Nurse Specialist (CNS) triages all referrals to the MBU. The triage process includes gathering information on the physical health of the baby.
- All referral information is documented on a Mental Health Triage form in webPSOLIS.
- Other contact with the patient /referrer is documented as a service event in PSOLIS.
- If the baby is unwell with a potential infectious condition the management of the baby will be discussed with the Infection Prevention and Control team and appropriate quarantine periods implemented as per the FSFHG Transmission Based Precautions policy and procedure.

A baby may be admitted as a boarder to the MBU up to 12 months of age if the baby is non-ambulant. Babies less than 12months who are ambulant will not be admitted.

3.2. Identification

- All babies registered as boarders must wear two patient identification armbands: one arm and one leg at all times as per the FSFHG Patient Identification policy and procedure.
- Patient identification bands must be checked prior to providing care to the baby boarder.
- Photographic identification may be considered for the baby boarder where there are concerns regarding the baby's skin integrity or if requested by the mother.

3.3. Physical health requirements

- On admission to the ward:
 - Nursing staff will document the baby's temperature in the baby boarder's medical record and handover to the MBU resident medical officer (RMO).
 - The MBU RMO will conduct a well-baby check of the baby and discuss with the paediatric team if indicated.
 - The well-baby check will ideally be conducted within six hours of admission but no later than 24 hours.
 - The well-baby check is documented on MBU Baby Assessment form.
- Weight is recorded by nursing staff on admission and then weekly using the:
 - MBU Baby Infant History Form
 - Girl/Boy growth charts

- MBU Weight chart
- A neonatal review and assessment may be considered for babies of breastfeeding mothers on multiple psychotropic medications.

3.4. Escalation of care

- New medical issues identified by the parents or MBU staff will be reported to the MBU RMO.
- A set of physiological observations will be completed and reported to the medical officer to discuss with the paediatric team if required.
- The MBU RMO will consult with the paediatric/neonatal/Infection Prevention and Control teams for advice when required.
- In hours 08:00–16:30hrs the MBU RMO will determine if the baby should be taken to the Emergency Department (ED) or to the family General Practitioner for assessment.
- Between 16:30-08:00hrs MBU staff will escalate care to the Psychiatric Registrar on call.
- If the baby is for review in the ED, the mother may attend the ED with her baby. The requirement for MBU nurse escort is determined on a case by case basis based on the clinical presentation of the baby. Clinical handover will be provided by the MBU Team during business hours and the Psychiatric Registrar/HOOT CNM after hours.
- Neonatal or paediatric code blue emergencies will be managed as per the FSH Code Blue – Medical Emergency policy.

3.5. Sleeping

- Prior to the admission (during the triage process) the mother will be informed that care will be provided in line with the Department of Health [Safe Infant Sleeping Policy](#).
- On admission nursing staff will discuss sleeping patterns with the mother.
- The mother will be provided with written information on [Safe Sleeping](#)
- Necklaces, bracelets, dummies on chains must be removed as they pose a choking hazard. This will occur following a discussion with the mother explaining the risks. If the mother refuses to remove these items nursing staff will:
 - immediately inform the CNS/ multidisciplinary team (MDT)
 - document in the patient's medical record
 - complete a clinical incident notification in Datix CIMS
- All mothers are provided with clear communication about their baby's sleeping or rest times/routine and education and support to record the baby's sleeping and feeding routine on the MBU Sleep and Feeding chart during their admission.
- Use of the nursery will be discussed and determined by the MDT based on clinical grounds.
- All care provided to the baby boarder will be communicated to the mother during bedside handovers.

3.6. Feeding

- On admission nursing staff will discuss feeding patterns with the mother
- Concerns regarding feeding routines will be escalated to the medical officer and documented in the baby's paper file.
- Referrals may be made to the Lactation Consultant using E-referral.
- If there are concerns regarding the baby's growth, feeding or reflux the baby may be referred to the Paediatric Rapid Assessment and Management (PRAM) Clinic by contacting the Paediatric Registrar by phone to discuss. If the referral is deemed appropriate an E-referral can then be made.

3.7. Medication administration

- At admission all currently prescribed or over the counter medications the parents have been administering to the baby will be handed to the nursing staff and treated as patients own medications.
- The MBU RMO completing the well-baby check will review medications and prescribe on the Paediatric Hospital Medication Chart as required.
- New medications prescribed while the baby is in hospital will be charted on the medication chart and will comply with State Medicines Formulary restrictions.
- Ward staff will administer prescribed medications to the baby.
- Non-imprest items will require dispensing at a community pharmacy. These items will be sourced by the patient's partner/parent/carer/agent and subsequently handed over to ward staff for storage and administration.
- Imprest items supplied by the pharmacy are limited to:
 - Paracetamol liquid, oral
 - Nystatin, oral drops
 - Chloramphenicol, ophthalmic
 - Hydrocortisone, topical.
- The MBU Pharmacist will provide minimal support for patients on the above medications. Medications outside this list may require liaison with the Paediatric Department and if prescribed liaison with the paediatric ward Pharmacist.

3.8. Management of the Nursery

- The nursery door must remain locked at all times and no obstructions to be placed on the door or viewing window.
- The CNS, Clinical Nurse (CN)/Shift Coordinator and Consultant Psychiatrist will have a daily nursery huddle on the afternoon shift to plan nursery allocations.
- Baby boarder duration/use of the nursery will be reviewed daily by the MDT. Prioritisation of allocation will be determined based on clinical need including planning for discharge.
- Only four babies are allowed in the nursery at any one time:
 - One baby in each pod

- Two babies in the main nursery area.
- In the event that > four babies require nursing care overnight:
 - During business hours the CN/Shift Coordinator will escalate to the CNS and request an urgent review of the nursery by the MBU medical team
 - Out of Hours the CN/Shift Coordinator will escalate to the HOOT CNM to ensure appropriate staffing ratios to provide safe care to baby boarders.
 - Consider the use of reverse monitors in the mother's room.
 - When considering the nursing station ensure the environment is safe and free from hazards.
- Baby boarders must be observed according to the observation level agreed by the MDT while they sleep or rest.
- Nursing staff must physically enter the nursery when completing observations.
- Observations must be documented on the baby's Sleep and Feeding Chart.

3.9. Nursery equipment

- Cots and bassinets must comply with Sudden Infant Death Syndrome (SIDS) Safe Sleeping guidelines and must be in good repair including mattresses
- Cots, bassinets and bedding will be cleaned as per the Environmental Cleaning policy after each use.
- Cot sides must be up and securely locked into place when in use.
- Cots and bassinets must be arranged to allow unobstructed access to the nursery.
- An operational monitor must be available at all times.
- Prams, pushers and rockers must not be used unsupervised or as a sleeping environment for the baby boarder.
- If a baby will only settle when in a pram nursing staff must remain with the baby until the baby is asleep and then transfer into a safe cot or bassinet.

3.10. Documentation

- Care provided to the baby will be documented onto a hard copy Mother and Baby Assessment Form
- On discharge all documentation for the baby is scanned into the baby's digital medical record and filed under Correspondence in the baby's DMR
- Details about the baby's time on the ward will be documented in the 'additional information' section of the mothers Notification and Clinical Summary (NaCS) and a copy sent to the nominated General Practitioner.

4. Compliance/performance monitoring

The MBU CNS will monitor compliance with this procedure via medical record auditing and routine review of clinical incident reporting.

5. Related policy documents

- WA Health:
 - [Safe Infant Sleeping Policy](#) (MP 0106/19)
- SMHS:
 - [Boarders in Hospital](#) (SMHS :25)
- FSFHG:
 - Patient Identification (FSFH-HW-POL-0034)
 - Medication Administration (FSFH-HW-POL-0022)
 - Observation and Response Chart (FSFH-HW-POL-0032)
 - Transmission Based Precautions (FSH-IPM-POL-0008)
 - Mother and Baby Co-sleeping- Management and Prevention (FSH-HW-POL-0038)
 - Environmental Cleaning (FSFH-HW-POL-0002)

6. Related standards

- NSQHS Standards:
 - Partnering with Consumers
 - Preventing and Controlling Infection
 - Medication Safety
 - Communicating for Safety
 - Recognising and Responding to Acute Deterioration

7. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Authorised By	Revision due
1	06/2019	Clinical Nurse Specialist, Mother Baby Unit	MHS SQR Committee NMPGC	FSFHG Policy Committee	06/2022
2	04/2021	Clinical Nurse Specialist, Mother Baby Unit		SQR Committee, Service 5	04/2024
3	06/2024	Clinical Nurse Specialist, Mother Baby Unit	DTC	SQR Committee, Service 5	07/2028